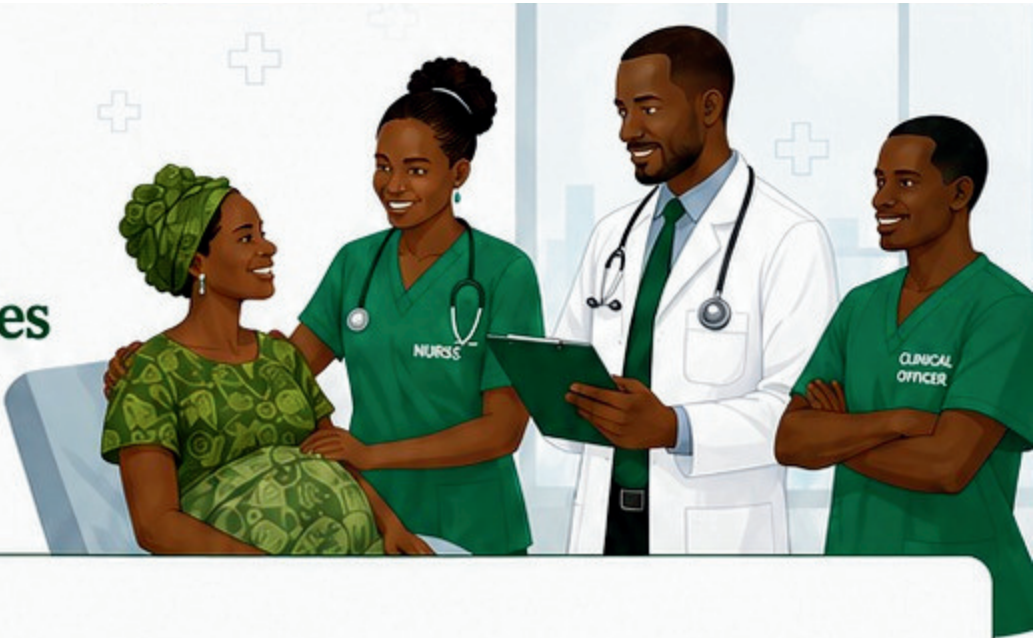


Enhancing Maternal Survival by Operationalizing the National Post-Abortion Care (PAC) Guidelines for County Level Impact



EXECUTIVE SUMMARY

Maternal mortality remains a critical public health challenge in Kenya. Post-Abortion Care (PAC) is recognized by the Ministry of Health as one of the six foundational pillars of the national maternal and newborn health model. While the national government provided a standardized framework via the 2019 PAC Pocket Guide for Health Care Providers, health service delivery is fully devolved.

This brief outlines the strategic gaps in regarding PAC implementation at county level and offers actionable recommendations to align county health systems with constitutional mandates, improve UHC coverage, and save women's lives.

PROBLEM STATEMENT

- **The Devolved Mandate:** Under the Constitution of Kenya (2010), county governments are entirely responsible for the delivery of primary and secondary healthcare services
- **The Constitutional and Statutory Right:** Article 43(2) of the Constitution and the Health Act (2017) dictate that emergency medical treatment is a fundamental right. The national PAC guideline explicitly clarifies that conscientious objection cannot be exercised by medical staff in emergency situations
- **The Local Reality:** Despite these strong legal protections, women in our county continue to face preventable morbidity and mortality due to incomplete abortions. This is driven by an inadequate supply of essential commodities, gaps in healthcare worker training, and institutional barriers rooted in personal bias or stigma

NATIONAL PAC GUIDELINE: THREE CORE PILLARS



POLICY GAPS AT THE COUNTY LEVEL

- **Supply Chain Inconsistencies:** Frequent stockouts of first-trimester uterine evacuation commodities, specifically MVA kits, Lignocaine, and Medical Abortion drugs, at Sub-County hospitals and local dispensaries
- **Capacity Gaps Among Mid-Level Providers:** Frontline nurses and clinical officers frequently lack structured orientation on the specialized counseling and clinical assessment tools dictated by the guidelines
- **Lack of Institutional Accountability:** Systemic failure by County Health Management Teams to enforce the Health Act's restriction on conscientious objection in emergency setups, leading to unnecessary delays or service denials

PRIORITY ACTIONS FOR COUNTY LEADERSHIP

Priority Area	Recommended Action	Expected Outcome/Target
 Financing and Commodity Security	Establish a dedicated, ring-fenced budget line for reproductive health commodities within the County Integrated Development Plan, Annual Development Plan, and annual county budgets	Ensure a consistent (100%) supply of Manual Vacuum Aspiration kits and Medical Abortion medicines in all Tier 2 and Tier 3 public health facilities
 Workforce Capacity Development	Partner with professional associations, reproductive health networks, and civil society organizations to provide regular training and sensitization on the Kenya Post-Abortion Care Pocket Guide and National PAC Standards. Integrate PAC into Continuous Professional Development programmes for clinical officers, nurses, and other frontline health workers	Increase the number of health workers who are competent and confident in providing quality, respectful, and evidence-based post-abortion care services
 Accountability and Oversight	Mandate the County Health Management Team to conduct routine maternal health and post-abortion care audits, including monitoring compliance with Section 12(2) of the Health Act, 2017, which prohibits denial of emergency treatment. Establish confidential, non-punitive systems for reporting commodity shortages, staffing gaps, and service delivery challenges	Strengthen accountability, improve compliance with national standards, and prevent avoidable denial or delays in emergency post-abortion care services
 Universal Health Coverage Integration	Include comprehensive emergency post-abortion care services within county Social Health Authority implementation plans and benefit packages to ensure financial protection for women and girls requiring emergency reproductive health services	Reduce out-of-pocket expenditure, improve equitable access to life-saving post-abortion care, and advance Universal Health Coverage

POLICY RECOMMENDATIONS FOR COUNTY LEADERSHIP

- Strengthen procurement and uninterrupted commodity supply
- Train and mentor frontline providers on PAC clinical protocols and counselling
- Institutionalize routine supportive supervision and quality improvement
- Enforce emergency treatment obligations under the Constitution and Health Act
- Integrate PAC Indicators into county performance monitoring and budgeting

EXPECTED OUTCOME



Reduced maternal deaths



Improved emergency obstetric care



Stronger Universal Health Coverage



Higher quality respectful maternity services